intersex NIGERIA
CENTER FOR HEALTHCARE DEVELOPMENT & YOUTH EMPOWERMENT

Understanding the Menstrual Health Realities and Gaps of Intersex Persons in Nigeria.

A report by Intersex Nigeria
(Center for Healthcare Development and Youth Empowerment)



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Contact Information

Intersex Nigeria
(Center for Healthcare Development and Youth Empowerment)

Lagos and Abuja, Nigeria

info@intersexnigeria.org

Visit our website

www.intersexnigeria.org

Social Media

@Intersex_nigeria_

@IntersexNigeria

@IntersexNigeria

Intersex Nigeria

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Foreword

Menstrual health is increasingly recognised as a cornerstone of sexual and reproductive health, human dignity, and gender equity. Yet, despite global progress, important gaps remain in ensuring that all people who menstruate are acknowledged and supported within health systems. Among the most invisible in these conversations are intersex individuals, whose unique needs and lived experiences are often silenced by rigid binary frameworks, stigma, and systemic neglect.

This research breaks new ground by shedding light on the menstrual health realities of intersex persons in Nigeria, a subject that has remained largely unexamined. By centering intersex voices and engaging healthcare providers, the study exposes the layered barriers of limited awareness, stigma, inadequate training, and policy exclusion that prevent intersex individuals from accessing equitable care. It also highlights the psychosocial dimensions of menstruation, revealing the secrecy, shame, and isolation that many intersex people endure, alongside the resilience of those who find support in family or community.

The findings are not only timely but urgent. They provide clear evidence that intersex inclusion in menstrual health is both a human rights imperative and a public health necessity. The recommendations offered, spanning education, healthcare training, policy reform, and psychosocial support, present a roadmap for building a health system that is inclusive, respectful, and responsive to the needs of all.

This study will serve as a critical resource for policymakers, healthcare providers, advocates, and scholars committed to advancing health equity. Most importantly, it amplifies the voices of intersex individuals who, for too long, have been excluded from conversations about their own bodies and rights.

We hope that this research will inspire national dialogue and meaningful action toward an inclusive future where every person who menstruates is supported, respected, and empowered.

Acknowledgement

Intersex Nigeria (Centre for Healthcare Development and Youth Empowerment) extends heartfelt appreciation to every intersex person and healthcare professional who contributed to this research. Your courage, openness, and willingness to share lived experiences are the foundation of this work. By sharing your stories and insights, you have broken silences and brought visibility to an issue that has too often been ignored.

We wish to express our deepest and most profound gratitude to our primary funders, Fos Feminista and the Agence Française de Développement (AFD), whose support made the Sang Pour Sang project a reality. We are immensely grateful for your unwavering partnership and for recognizing the critical need for intersex-led research. Your commitment to reproductive justice and the rights of marginalized bodies provided the necessary foundation for this project to thrive. By investing in this work, you have empowered us to challenge systemic inequities and center the voices of those often pushed to the margins of healthcare and human rights.

The support of Fos Feminista and the Agence Française de Développement (AFD) has been instrumental in bridging the gap between research and social impact. We appreciate your dedication to inclusive development and your belief in a future where healthcare is a right, not a privilege. Through your funding, we have been able to produce a body of evidence that we believe will serve as a catalyst for policy change and social justice in Nigeria for years to come.

Through the Sang Pour Sang project, Fos Feminista and the Agence Française de Développement (AFD) has done more than fund a report; you have provided a platform for truth-telling and a catalyst for systemic change within Nigeria's healthcare and social landscapes.

We are deeply grateful to our consultants, Parton Titus and Moses Danjuma Kusko, for their tireless commitment, passion, and expertise. Their dedication reflects the power of research not just to inform, but to challenge stigma, shift narratives, and drive meaningful social change.

Our sincere thanks go to the intersex persons who believed in this project and shared their stories to ensure authenticity of the report, your support has been instrumental in making this project possible. Your commitment to the intersex movement has amplified intersex voices in Nigeria and reaffirmed that inclusion and equity must remain at the heart of health and human rights. Through your lived experience and unwavering dedication to advancing the rights, dignity and well-being of intersex persons continue to inspire and shape meaningful change within and beyond our commitments.

Finally, to the entire team at Intersex Nigeria, thank you for your resilience, advocacy, and determination that made this work a reality. This research stands as a collective achievement, one that brings us closer to a Nigeria where intersex persons are seen, respected, and empowered.

Executive Summary

This study explores the menstrual health needs and experiences of intersex individuals in Nigeria and assesses the knowledge, attitudes, and practices of healthcare workers (HCWs) in providing intersex-inclusive care. Despite growing attention to menstrual health globally, intersex people remain largely excluded from research, discourses, policies, and programs. This invisibility results in unmet health needs, psychosocial challenges, and systemic exclusion within Nigeria's sexual and reproductive health (SRH) landscape. While most healthcare workers had heard of intersex, only 15% had received any formal training, and just 6% had ever accessed clinical protocols on intersex care. Intersex individuals also reported learning about menstruation informally, often through peers or family, with little or no accurate or inclusive education.

Both intersex participants and healthcare workers identified stigma and judgment as major barriers. With 77% of HCWs citing stigma as a key challenge, the study reveals how binary SRH frameworks and societal prejudice reinforce invisibility and discourage intersex people from seeking care. Nearly half of HCWs identified policy gaps as a barrier, with many noting that intersex health is absent from SRH policies and programs. Without formal inclusion, service delivery remains inconsistent and exclusionary. Intersex individuals described secrecy, shame, and internal conflict around menstruation, often linked to rigid gender norms. While some had family support, most lacked community networks or peer groups, increasing vulnerability to isolation and poor mental health outcomes.

The study concludes that intersex individuals in Nigeria face layered barriers from inadequate education and exclusionary policies to provider stigma and systemic neglect. At the same time, healthcare workers express readiness to improve their capacity to provide inclusive care, highlighting demand for continuing medical education, mentorship from specialists, and national clinical guidelines.

This research provides the first targeted evidence on intersex menstrual health in Nigeria, highlighting urgent gaps in knowledge, policy, and service delivery. It shows that inclusive menstrual health is both a human rights imperative and a public health necessity. Addressing stigma, strengthening provider capacity, and embedding intersex realities in policy will ensure that intersex individuals are no longer invisible in Nigeria's health system.

Acronyms

AIS – Androgen Insensitivity Syndrome
CAH – Congenital Adrenal Hyperplasia
HCW – Healthcare Worker
IPA – Interpretative Phenomenological Analysis
KII – Key Informant Interview
MRKH – Mayer–Rokitansky–Küster–Hauser Syndrome
SRH – Sexual and Reproductive Health
UNFE – United Nations Free & Equal
UN OHCHR – United Nations Office of the High Commissioner for Human Rights
WASH – Water, Sanitation, and Hygiene
WHO – World Health Organization
CHEW-Community Health Extension Worker

01

Introduction

Background of the Study

Intersex is an umbrella term used to describe individuals born with physical and biological sex characteristics such as chromosomes, hormones, gonads, or genitals that do not fit typical binary notions of male or female bodies. These variations may be apparent at birth, become visible during puberty, or remain undetected unless detected through medical testing. Over 40 intersex variations have been documented, and according to the United Nations Free & Equal campaign, between 0.05% and 1.7% of the global population is born with intersex traits (UNFE, 2017, HRW, 2024). Despite being a natural part of human diversity, intersex individuals often face significant stigma, discrimination, and human rights violations. These include non-consensual and harmful medical practices/surgeries, limited access to accurate information, limited legal recognition, exclusion from anti-discrimination laws, barriers to obtaining precise identification documents, inadequate legal redress mechanisms, denial of the right to self-identify, and systemic exclusion from health services (HRW, 2017). In many societies, including Nigeria, deeply rooted binary norms of sex and gender contribute to the invisibility and further marginalization of intersex persons, especially within health systems and policy frameworks that do not acknowledge their existence.

A key area of concern is menstrual health, which is traditionally framed within the experiences of cisgender women and girls. This shows the lack of inclusion of intersex persons in menstrual health policies and education. Research on menstrual health policies in Nigeria primarily focuses on adolescent girls and women (Olasehinde, 2025). This framing often excludes intersex individuals who were assigned male at birth who menstruate or experience menstrual-related health issues. This exclusionary framing leads to a significant gap in tailored menstrual education, products, and healthcare for many intersex people in Nigeria, often resulting in shame, confusion, mental health challenges, and diminished bodily autonomy.

Menstrual health is a state of complete physical, mental, and social well-being concerning a person's menstrual cycle. It includes access to accurate information, affordable menstrual products, clean and safe sanitation facilities, supportive healthcare services for menstrual concerns, including pain and irregularity, and an environment free from stigma. Global frameworks now recognise that menstrual health applies to all people who menstruate, not just women and girls, and call for inclusive approaches that respect diversity in gender and sex characteristics (Åkerman, 2024).



For intersex individuals, menstrual health needs are particularly complex. Depending on the specific intersex variation, individuals may have reproductive anatomy that supports and undergoes menstruation like in cases of Congenital Adrenal Hyperplasia or 46, XX ovotesticular differences, or may not menstruate at all like those with Complete Androgen Insensitivity Syndrome,
Mayer Rokitansky Küster Hauser Syndrome, or Turner Syndrome (Mazur et al., 2009)

Statement of the Problem

Menstrual health in Nigeria has gained increased visibility in recent years, but this attention has not translated into inclusion for all who menstruate. Intersex individuals remain excluded from national conversations, policies, and programmatic responses on menstrual hygiene management. The absence of research and reliable data further exacerbates their exclusion.

Healthcare practitioners who play a critical role in ensuring menstrual health often have limited or no exposure to intersex-specific health needs. Consequently, intersex persons who menstruate are usually met with confusion, misdiagnosis, or outright stigma and inadequate care when accessing health services. rights-based menstrual health programming.

These gaps undermine their rights to safe, dignified, and affirming menstrual healthcare.

This research addresses the urgent need to understand and document the menstrual health experiences of intersex individuals in Nigeria, while also evaluating the knowledge, attitudes, and practices of healthcare providers on intersex menstrual health needs. The findings from this research aim to inform policy, strengthen health providers' capacity and support the development of inclusive and rights-based menstrual health programming.

Objectives of the Study

Main Objective

To explore the menstrual health needs of intersex individuals in Nigeria and assess the preparedness of healthcare professionals to provide inclusive and responsive care for intersex people.

Specific Objectives

- To document the lived menstrual health experiences of intersex individuals in Nigeria.
- To assess the awareness and responsiveness of healthcare providers toward intersex menstrual health care.
- To identify systemic, institutional, and policy gaps related to intersex -inclusive menstrual health services.
- To provide actionable and evidence-based recommendations for improving intersex inclusion in menstrual health policies and practices

Research Questions

I. What are the diverse lived experiences of menstruation among people with innate variations in sex characteristics in Nigeria?

II. How informed are healthcare providers regarding the menstrual health needs of intersex individuals in Nigeria?

III. What are the key gaps in existing healthcare service delivery and public policies that hinder adequate and inclusive menstrual health support for intersex individuals in Nigeria?

IV. What specific and actionable recommendations can be developed to enhance intersex inclusion within menstrual health policies, practices, and service delivery frameworks in Nigeria?

Significance of the Study

This study will provide targeted data on intersex menstrual health in Nigeria, offering a unique opportunity to:

- Influence the inclusion of intersex SRH policies and healthcare training curricula.
- Empower intersex groups with advocacy tools and data for advocacy
- Raise awareness among key stakeholders on the overlooked needs of intersex individuals.



02

Literature Review

Understanding Intersex Variations and Healthcare Needs

Intersex individuals are born with innate sex characteristics that do not fit typical definitions of male or female bodies. These can include variations in chromosomes, gonads, hormones, or genitalia (UNFE, 2017). These biological variations can be evident at birth or become noticeable later in life, such as during puberty (Woodward & Neilson, 2013). More than 40 intersex variations have been documented globally, including Congenital Adrenal Hyperplasia (CAH), Androgen Insensitivity Syndrome (AIS), Klinefelter Syndrome, Turner Syndrome, and Mayer–Rokitansky–Küster–Hauser (MRKH) Syndrome, according to the intersex advocacy organisation interACT (Eleesha 2024).

These variations can result in a broad spectrum of reproductive and hormonal development, which affects sexual and reproductive health outcomes, including menstruation.

Studies have shown that clinical management of intersex individuals, especially in Global South contexts, often involves early gender assignment and irreversible genital surgeries without informed consent (Boyle et al., 2005; Rebelo et al., 2008). These procedures, while medically justified as "normalising," have long-term physical and psychological consequences, including trauma, identity confusion, and bodily dysphoria (Chadwick et al., 2005).

Globally, intersex individuals experience discrimination in medical, legal, and social contexts (Human Rights Watch, 2017).

In particular, non-consensual and medically unnecessary surgeries performed on intersex infants and children have been condemned by UN human rights bodies as violations of bodily autonomy (UN OHCHR, 2015). Health systems often operate on binary assumptions of sex, rendering intersex experiences invisible or pathologised.

Menstrual Health and Inclusive Framework

The World Health Organisation (WHO) calls for the recognition and framing of menstruation as a health issue (not a hygiene issue) with physical, psychological, and social dimensions, and one that needs to be addressed in the perspective of a life course from before menarche to after menopause. WHO calls for menstrual Health to be recognised,

framed and addressed as a health and human rights issue, not a hygiene issue; menstrual health means that women and girls and other people who menstruate, have access to information and education about menstruation; Have access to menstrual products they need; water, sanitation, and disposal facilities; have access to competent and empathic care when needed; have access to live, study and work in an environment in which menstruation is seen as positive and healthy not something to be ashamed of; and to fully participate in work and social activities. (WHO 2022).

In Nigeria, menstruation is often surrounded by taboos and silence. Many communities treat menstruating women as ritually unclean or restrict their activities. For example, older Nigerian women recount that in the past, mothers were often afraid to tell fathers when their daughters were menstruating for fear they would blame the girls for doing something wrong. Husbands have been known to avoid their wives or even refuse to eat food prepared by women during menstrual periods (Plan International USA, 2024).

These cultural attitudes, along with limited water and sanitation facilities, contribute to what observers call period poverty and pervasive shame around menstruation. A study reported that 23% of Nigerian adolescent girls missed school due to menstruation challenges (Nigeria Health Watch, 2025), a reflection of the lack of privacy, products, and supportive understanding in schools and homes.

These general menstrual taboos intersect with Nigeria's conservative views on sex and gender. Intersex variations are largely unknown to the public: traditional beliefs may brand intersex traits as curses or bad omens (Outright International, 2024).

There has been virtually no visibility or recognition of intersex people in Nigerian law or health policy. Only very recently, in 2024–2025, has the Federal Ministry of Health begun to "initiate a national conversation" on intersex inclusion. Officials have admitted that no national data exist on intersex prevalence and that nearly all cases go undocumented or are only identified in tertiary hospitals. To date, there is no standalone policy for intersex health (thenationonline, 2025).

Menstrual health is increasingly recognised as a public health and human rights issue. The World Health Organisation defines menstrual health as a state of physical, mental, and social well-being concerning the menstrual cycle, not merely the absence of disease or discomfort (WHO, 2022).

Recent research has documented the unique menstrual health experiences of trans and intersex individuals, including psychological distress, social isolation, and denial of services due to non-conforming gender presentation (Dubey et al., 2024; Muskan T, 2024). This shows the importance of intersectional and inclusive policies.

Menstruation in Intersex Individuals

Menstruation, often perceived exclusively as cisgender women's experience, is a complex and often distressing phenomenon among some intersex individuals. Depending on the specific intersex variation, intersex persons may experience regular bleeding (e.g., 46, XX individuals with Congenital Adrenal Hyperplasia) or none at all (e.g., AIS, Turner Syndrome) (Malouf et al., 2010) or irregular periods. In some rare cases, individuals assigned male at birth may present with uterine or vaginal tissue and report unexplained bleeding (Hutson et al., 2012).



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Those with AIS or MRKH syndrome typically have XX chromosomes but do not menstruate due to the absence of internal female reproductive organs. In some cases, people with ambiguous genitalia may experience vaginal bleeding without understanding its origin due to a lack of appropriate medical guidance (Lee et al., 2006).

However, studies examining menstruation specifically among intersex people are sparse. In existing qualitative studies, intersex individuals report confusion, isolation, and shame surrounding menstruation, especially when healthcare providers lack knowledge or sensitivity (Sanders & Carter, 2015). In low-resource settings like Nigeria, this invisibility is even more pronounced due to poor SRH (Sexual and Reproductive Health) infrastructure, rigid gender binaries, and high levels of stigma (Warne & Raza, 2008).

The diversity of intersex experiences challenges medical and policy frameworks that treat menstruation as uniform. Invisibility in menstrual discourse compounds confusion, shame, and lack of access to appropriate care. Studies note that intersex people often feel alienated from both women's health spaces and general healthcare systems (Jones et al., 2016).

There is a clear absence of data on how intersex people experience and manage menstruation in Nigeria, and those who menstruate often do so in secrecy due to stigma, lack of information, and fear of being stigmatised and discriminated against. Without a framework for care, their unique experiences remain undocumented and misunderstood.

Healthcare Providers and Intersex-Inclusive SRH Services

Healthcare professionals often lack adequate training on intersex health, particularly regarding SRH issues like menstruation, fertility support, and hormonal care (Sanders et al., 2015). Studies report clinical mismanagement, insensitive communication, and reinforcement of binary sex norms (Al-Jurayyan, 2011; Ediati et al., 2015a). In Nigeria, this is further compounded by limited exposure to intersex issues in medical curricula (Mungadi, 2015).

The Nigerian healthcare providers face several structural barriers, including high rates of home deliveries, underfunded SRH services, and cultural taboos around menstruation (Austin et al., 2015; Adelaja, 2011). This has made the knowledge, attitudes, and practices of healthcare professionals very important in shaping intersex people's access to safe and affirming SRH services.

Few Nigerian clinics have protocols for intersex health. The research by Intersex Nigeria found that 85% of doctors and nurses in a sample of 50 hospitals had encountered an intersex birth in the last five years, yet lacked any guidelines or training (Intersex Nigeria 2025). General menstrual health services like gynaecological examination, pain and hormonal management are not designed with intersex bodies in mind. As a result, intersex individuals are either misdiagnosed or discouraged from seeking care altogether.

The absence of intersex-inclusive SRH protocols not only compromises health outcomes but also contributes to fear, stigma, and delayed care-seeking among intersex people. Developing a standard operating procedure and integrating intersex health into medical training are critical steps toward ensuring equitable access to menstrual and SRH services.

> ***"Depending on the specific intersex variation, intersex persons may experience regular bleeding (e.g., 46, XX individuals with Congenital Adrenal Hyperplasia) or none at all (e.g., AIS, Turner Syndrome) (Malouf et al., 2010) or irregular periods. In some rare cases, individuals assigned male at birth may present with uterine or vaginal tissue and report unexplained bleeding (Hutson et al., 2012)."***

Policy Gaps and Barriers to Intersex Menstrual Inclusion in Nigeria

Menstrual health education and policy frameworks in Nigeria are typically geared to only target cisgender girls and women, and often delivered late or not at all due to taboo. Intersex individuals may be completely excluded from menstrual health discussions or menstrual health materials. In a study, Nigerian intersex men reported having no idea why they were bleeding each month, initially thinking it was hematuria (blood from disease) or parasites (Sani et.al; 2020). Neither their families nor schools taught them to expect menstruation. Similarly, intersex women who do not menstruate often only discover the reason in adulthood. This ignorance is a critical barrier; without accurate information, intersex people cannot manage menstruation or amenorrhea, and may feel abnormal or frightened.

Nigerian society strongly enforces the gender binary. An intersex person raised male who begins bleeding each month faces extreme shame and fear (Sani et.al, 2020). An intersex woman (or girl) who never menstruates may feel isolated, thinking something is wrong with her body. In both cases, people often hide the issue rather than seek help. This stigma-driven silence means families and communities do not provide guidance or resources.

Menstrual health policy in Nigeria has improved significantly in recent years, particularly through the implementation of the National Strategy for Adolescent and Youth Health and the Water, Sanitation, and Hygiene (WASH) programs. No Nigerian government policy explicitly addresses intersex health, nor do SRH programs include indicators or guidelines for intersex bodies.

This creates multiple systemic barriers, like distribution programs for menstrual hygiene products that target only girls and women. The absence of legal recognition for intersex people means their SRH rights are often ignored in legal protections.

In a country where stigma and misinformation remain rampant, the lack of a clear legal and policy framework enables continued medical neglect and social exclusion of intersex individuals (Chow-Quesada 2023). Like other poor women, intersex individuals in Nigeria may lack money for quality menstrual products or reliable sanitation in schools or workplaces. Intersex people who do not present as women (e.g assigned male at birth) might also feel discomfort or further stigma when obtaining products labelled for "women." While no specific data exist on intersex use of menstrual products, it is clear they share in the general period poverty that afflicts many Nigerian girls (NigeriaHealthWatch,2025). When products or facilities are scarce, the needs of intersex menstruators are even more likely to be ignored.

Despite increased global awareness of menstrual equity, most interventions target cisgender girls and women, excluding other menstruators such as intersex individuals. While foundational studies on birth and care practices in Nigeria provide insight into broader maternal health challenges, they do not include people with intersex traits (Envuladu et al., 2013).

Similarly, surgical and clinical studies focus on early gender assignment, overlooking the psychosocial and menstrual health needs that emerge in adolescence or adulthood (Özbey et al., 2004; Al-Jurayyan, 2010).

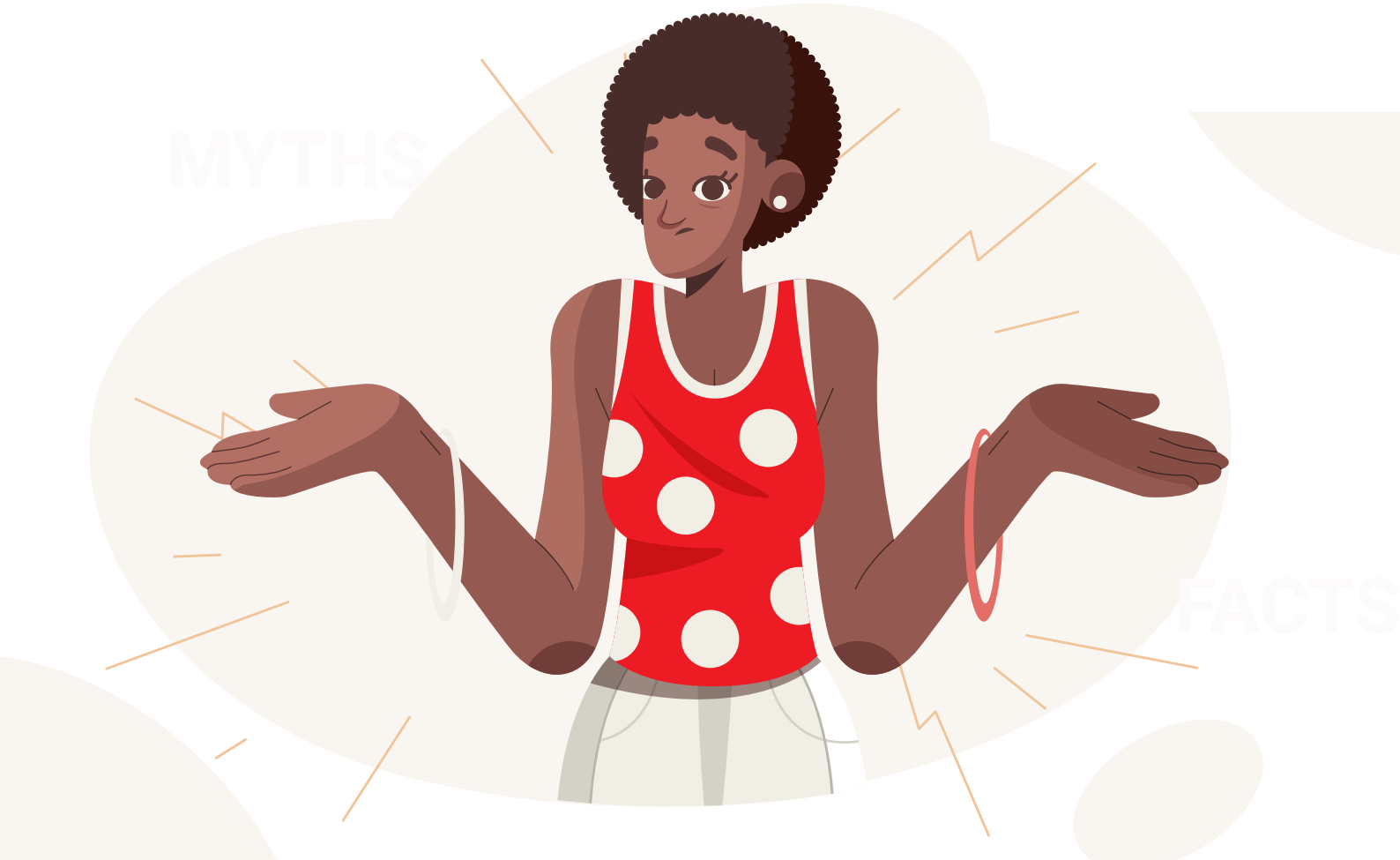
This gap supports the importance of the present study, which seeks to centre intersex voices and experiences in menstrual health discussions, particularly in a high-stigma environment like Nigeria.



Intersex and Health Access Challenges

Human Rights activists and other advocacy organisations have highlighted the marginalisation of intersex individuals in health systems globally. In Nigeria, there's no government-led data collection or programming for intersex persons. A systematic review of the health and healthcare inequalities for people with innate variations in sex characteristics suggests that intersex people are more likely than the general population to report unfavourable experiences of accessing healthcare, including poor communication from health professionals and dissatisfaction with the treatment and care received. Health professionals commonly accept male or female binary sex as the norm and express surprise when they learn of intersex variance, but feel unsure of how to accommodate the specific health needs of these individuals.

Their health needs are often poorlyunderstood by health professionals due to limited knowledge and understanding. (Zeeman & Aranda, 2020) The stigma of having atypical genitalia or reproductive functions contributes to psychological distress among intersex individuals. These include anxiety, depression, self-harming behaviours, and suicidal ideation (Schützmann et al., 2009). When compounded with menstrual-related confusion or pain, the distress intensifies, especially in conservative societies where menstruation is taboo. Intersex individuals have shared their struggles to reconcile their bodily functions with normative expectations, reflecting the same bodily surveillance experienced by cis women, but with added complexity and shame (Guntram 2013).



03 Methodology

Research Design

This study adopted a mixed-methods design to generate a comprehensive understanding of menstrual health experiences among intersex people and the readiness of healthcare systems to support them using both quantitative and qualitative data. The qualitative method is appropriate for investigating understudied or sensitive phenomena where limited empirical data exists.Given the invisibility and stigma surrounding intersex identities and menstrual health in Nigeria, a quantitative method was used for medical professionals, while a qualitative method, which allows deep contextual understanding, was used for healthcare professionals and intersex persons to understand healthcare system gaps, lived experiences and socio-cultural influences.

Qualitative methods, particularly Interpretative Phenomenological Analysis (IPA), have been instrumental in capturing the lived experiences of individuals (Smith & Osborn, 2015). IPA allows researchers to explore meaning-making around menstruation, stigma, and healthcare interactions. The importance of using participant-centred approaches in sensitive topics like gender and bodily autonomy cannot be overemphasised. Moreover, a study by Guest et al. (2006) shows that data saturation in qualitative research can be achieved with relatively small sample sizes when in-depth interviewing is used.

This study explored and interpreted the lived menstrual health experiences of intersex individuals and the perspectives of healthcare providers within the Nigerian healthcare and socio-cultural context.

Study Population

The study focuses on two primary populations: Intersex individuals aged 18 and above who reside in Nigeria, to answer questions related to menstruation, regardless of whether they menstruate or not. And healthcare professionals, including doctors, nurses, and midwives, who may have interacted with intersex individuals or are involved in menstrual health services.

Sampling Technique and Sample Size

A purposive sampling method was employed to identify intersex individuals through Intersex Nigeria support networks and peer referrals. Healthcare professionals were selected using referral networks, focusing on those involved in SRH, adolescent health, or menstrual hygiene management.

Due to the highly sensitive nature of intersex related healthcare and services in Nigeria and the prevailing stigma surrounding sexual and reproductive health, recruitment of intersex participants was particularly challenging. Twelve (12) intersex individuals participated in Key Informant Interviews (KIIs). While this number may appear small, it is consistent with exploratory research in marginalised populations where invisibility and fear of disclosure limit participation. The narratives obtained nonetheless provide rich, in-depth insights into the lived realities of intersex persons, offering valuable qualitative evidence for advocacy and future studies.

For healthcare workers, a mixed sampling approach was applied. A total of 65 healthcare workers across different cadres, including doctors, nurses, midwives, and community health extension workers (CHEW), were interviewed to capture perspectives on awareness, knowledge, and preparedness in providing menstrual health care for intersex individuals. Also an in-depth interview was conducted with 12 health care workers, one Doctor, and one nurse for each geopolitical zone. The mixed-methods design was therefore chosen not to achieve representativeness but to generate exploratory evidence, combining quantitative trends with qualitative depth to answer the research questions.

Data Collection Methods

The primary data source for this study was field-based data collected directly from healthcare providers and intersex individuals in Nigeria. This reliance on primary data was essential because of the limited availability of secondary data on intersex menstrual health within the Nigerian context. This study employed a mixed-methods approach to capture both breadth and depth of information on intersex menstrual health in Nigeria. Quantitative data were collected through structured surveys administered to healthcare providers across multiple cadres (doctors, nurses, midwives, and community health workers). The surveys were designed to measure awareness, training exposure, attitudes, and confidence levels in providing menstrual care to intersex individuals.

Qualitative data were gathered through Key Informant Interviews (KIIs) with both intersex individuals and healthcare providers. For intersex participants, interviews explored lived experiences of menstruation, access to information, psychosocial impacts, and barriers to care. For healthcare providers, KIIs probed perceptions, knowledge gaps, and systemic challenges. This approach allowed participants to share personal narratives, context-specific insights, and recommendations in their own words, thereby enriching the survey findings with lived realities.

The combination of survey data and in-depth interviews ensured triangulation, enhancing the validity of findings while capturing both numerical trends and nuanced experiences. Thisapproach is particularly critical for a marginalised group like intersex persons, whose realities are often invisible in mainstream data.

Ethical Considerations

Given the sensitivity surrounding intersex realities and SRH, strict ethical procedures were followed throughout the study. Informed consent was obtained from all participants after providing clear information on the purpose of the research,expected participation and the voluntary nature of involvement. Participants were informed of their right to withdraw at any time without penalty. We strictly maintained confidentiality and anonymity, as any form of identity was not captured for intersex individuals, and the data was securely stored.

Given the sensitivity of intersex experiences and realities, stringent ethical protocols were adhered to. All voice-recorded data were securely stored during analysis and deleted upon completion of the study. Furthermore, interviewers were trained in trauma-informed approaches and the need to provide support resources to participants in a situation where a respondent gets distressed during the interviews.

A guidance document was provided by Intersex Nigeria on respectful and inclusive engagement with intersex persons to support interviewers in conducting interviews and interactions with intersex persons in a manner that ensures sensitivity, accuracy, and respect in all processes.

04

Findings and analysis

The findings from this study provide an overview of the demographic characteristics, levels of awareness, training, attitudes, and perceptions regarding intersex inclusion in menstrual health services, including the lived experience of intersex individuals.

Demography of Health Care Providers Who Participated in the Survey

Professional Role

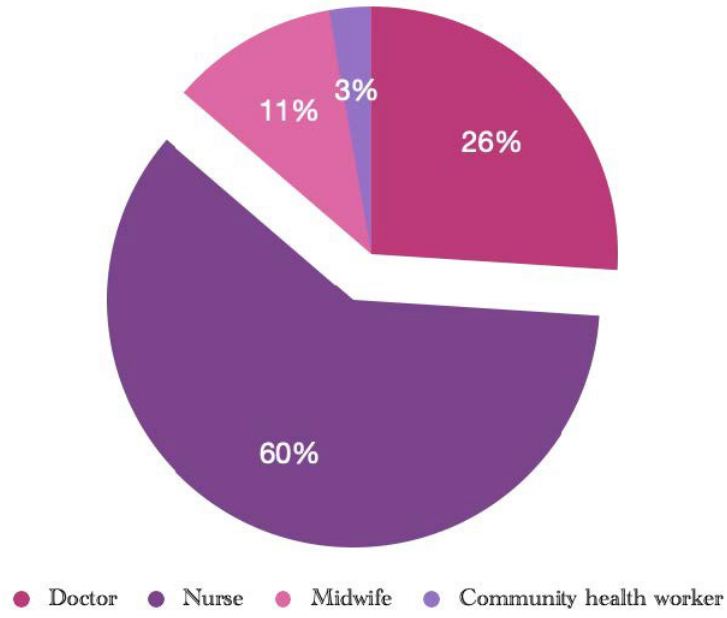
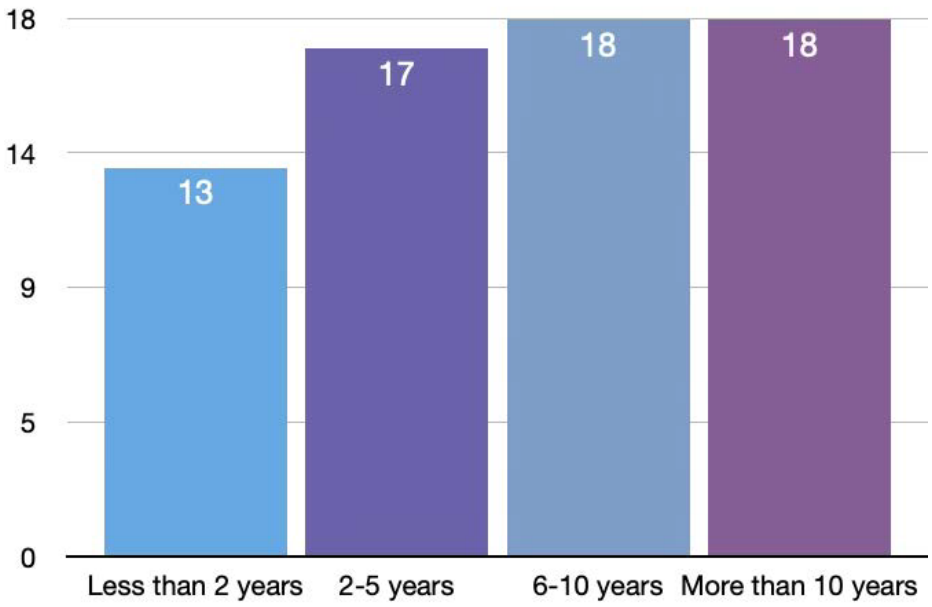


Figure 1: Healthcare workers by Professional Role

The professional composition of the healthcare worker respondents indicates that the majority of the respondents were nurses and doctors, making up over 86%, while midwives and community health workers make up about 14%. This distribution reflects the typical structure of Nigeria's frontline healthcare workforce, where doctors provide medical oversight and nurses play a central role in patient care and service delivery. This demographic spread is important, as it ensures that the findings capture perspectives from diverse cadres of healthcare providers who are directly or indirectly involved in menstrual and reproductive health services.

Years of Practice



Amongst the Healthcare workers that participated in the survey, 55% have more than 6years of professional experience, while 25% reported having 2–5 years of professional experience. Only 19% have less than 2 years' experience. This diversity ensures robustness of the findings, as it reflects the perspectives of healthcare workers from both relatively new professionals in the healthcare system and seasoned professionals with long-standing experience.

Figure 2: Healthcare workers by years of practice

Awareness and Knowledge of Intersex Menstrual Health

Awareness of intersex menstrual health realities was present among both healthcare providers and intersex participants, though the depth of understanding varied significantly. Among healthcare providers, survey results revealed that while awareness of the term intersex exists, detailed knowledge of menstrual health among intersex individuals was limited and often shaped by binary assumptions. Most providers had never received any formal training or exposure to intersex-specific reproductive health issues. This gap stems from the lack of inclusion of intersex topics in medical and nursing curricula, as well as the absence of clinical guidelines or continuing medical education on the subject.

As a result, healthcare workers often rely on conventional gender-based frameworks that fail to address the unique menstrual health experiences of intersex individuals. This limited understanding affects their ability to provide respectful, inclusive, and informed care when intersex patients seek support for menstrual or reproductive concerns.

For intersex individuals, knowledge about menstruation was equally limited, reflecting their exclusion from mainstream sexual and reproductive health education. Many reported confusion or fear when they first encountered menstrual bleeding or anxiety when they did not menstruate at all, despite societal expectations.

For some, menstruation was initially perceived as an illness or an abnormal event, while others struggled to interpret their bodily experiences due to the absence of accurate, non-stigmatizing information. This lack of tailored education and culturally sensitive guidance left them without the knowledge or resources needed to manage menstruation safely and confidently.

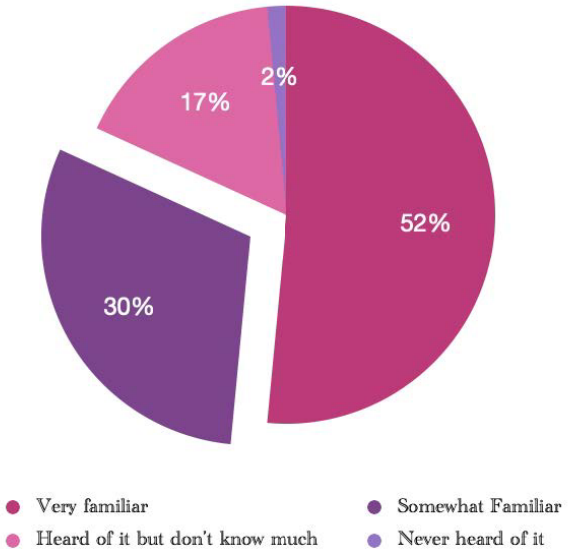


Figure 3: Healthcare workers familiarity with the term intersex

Out of the healthcare workers surveyed, a good number reported being very familiar.

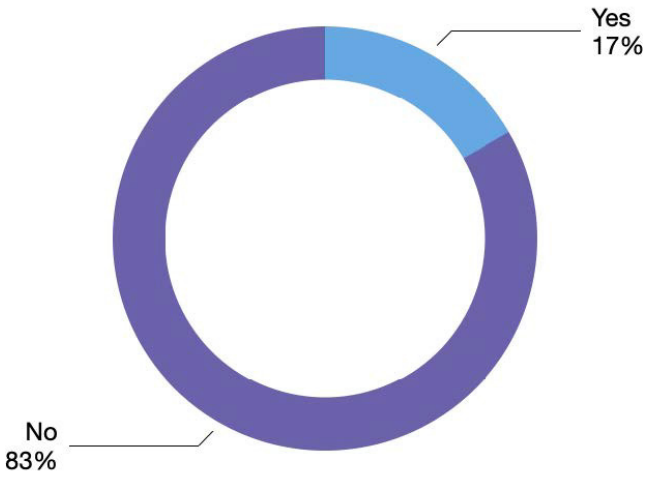


Figure 4: Healthcare workers trained on intersex health

Despite this general awareness, most healthcare workers admitted they had not received structured training on intersex health. Only 11 respondents (17%) had received training, compared to 55 (83%) who had not.

The survey showed that while most had heard of the term "intersex", only a minority possessed a detailed understanding of the menstrual health needs of intersex persons. Specifically, many acknowledged that intersex issues were rarely covered in their formal training, leaving them uncertain about appropriate care. This gap in training was echoed in an interview with one HCW who said:

> ***"We didn't learn much about intersex during training, so when such cases come, we are not sure how to respond."***

Access to Information and Menstrual Products

For intersex persons, awareness and access of menstruation information is shaped by gender-exclusive education. Many intersex persons recall being shocked and confused at first because schools taught that "only girls menstruate." One person shared:

> ***"When I first saw the blood, I thought I was sick. Nobody had told me it could happen to me."***

Intersex persons reported that information about menstruation was mostly acquired informally, mainly through friends or family, with little mention of intersex-specific realities in schools or health education. One person shared:t

> ***"Most of the information I got was from friends, not from teachers or doctors."***

Access to menstrual products also revealed exclusion. While many HCWs confirmed that standard products like pads are widely promoted, many intersex persons often found them uncomfortable and unsuitable. One person said:

> ***"I didn't know which products were right for me; pads felt uncomfortable but I didn't know alternatives existed."***

Considering the diversity of intersex bodies, and the unavailability of guidance on possible alternatives, such as menstrual cups or reusable options. Some intersex persons see affordability and accessibility as ongoing barriers. A respondent reported abandoning menstrual products entirely, stating:

> ***"I don't use any more… I don't."***

Intersex individuals experience both informational and material exclusion in relation to menstrual health. There are gaps in menstrual education, compounded by a lack of product diversity and affordability, which lead to unmet needs and improvised coping strategies, further marginalising intersex people from mainstream menstrual health interventions.

Interactions with Healthcare Providers

Healthcare systems are identified as a barrier to accessing safe and supportive menstrual health services for intersex individuals. The healthcare systems act as a barrier through systemic neglect such as absence of training, policy, and inclusive infrastructure for intersex related matters and through provider attitudes that perpetuate stigma and exclusion. Most intersex persons interviewed described negative or dismissive encounters with providers. Instead of receiving guidance or care, intersex persons often felt interrogated or invalidated. One person recounted:

> *"The nurse didn't understand my body; she just kept asking if I was really male or female instead of helping me."*

This reveals dismissive, and stigmatising encounters, with providers showing limited understanding of intersex realities.

> *"I once tried to ask about my cycle, but the doctor dismissed me, saying, 'That's not possible for someone like you."*

Psychosocial Impact and Stigma

Menstruation or its absence is described by individuals as carrying a heavy psychosocial burden, shaped by societal expectations that invalidates intersex bodies. Intersex persons spoke of 22 isolation, secrecy, and deep internal conflict, stemming from the pressure to conform to rigid gender norms. An intersex person mentioned:

> *"It feels like my body is betraying me because I don't fit what people expect."*

Survey data reinforced this, showing that while the majority of HCW believed intersex people can menstruate, a good percentage of health care workers were unsure. Only a very small percentage (5%) do not believe that intersex people can menstruate.

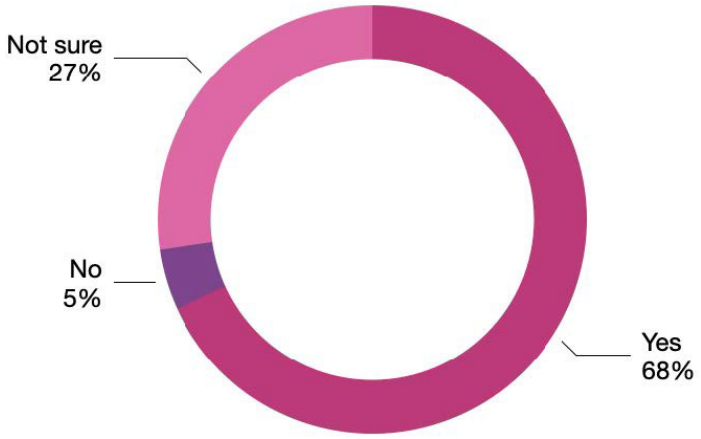


Figure 5: Healthcare workers perception of intersex menstruation

Another person said that:

> *"I kept it a secret for years, even from my family, because I didn't want to be seen as strange."*

These narratives reveal that intersex individuals experience layered stigma linked both to their identities and to menstruation itself. Judgment and discrimination also surfaced as a recurring thing as one intersex respondent admitted experiencing judgment:

> *"Yes…. (in response to being treated unfairly).*

With regards to support, some intersex individuals reported finding comfort and support within their immediate families. Unlike most intersex persons interviewed, one person identified parents as a source of support:

> *"It's my daddy and my mommy."*

While family support can act as a protective factor in some cases, the absence of broader peer and community-based support systems increases vulnerability to shame, secrecy, and poor mental health outcomes.

Attitudes and Service Delivery Gaps

Health care workers often framed menstruation in relation to girls and women only, reflecting the deeply entrenched binary orientations of SRH systems. Some acknowledged exclusionary practices, noting that intersex people rarely seek services, possibly due to fear of stigma.
One provider stated:

> *"We hardly see them in the facility. Maybe they don't come because they know we are not prepared for them."*

Such sentiments reveal both provider-level discomfort and broader systemic barriers that discourage intersex individuals from seeking essential health services.

HCW Encounters with an Intersex Patient seeking care for menstruation or related issues

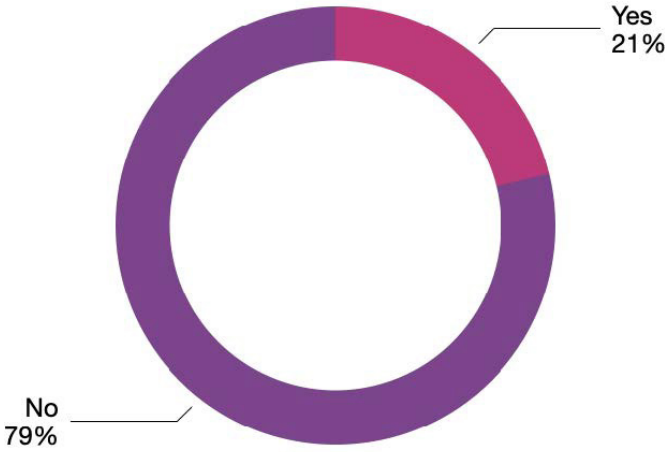


Figure 6: Shows Healthcare workers who have encountered intersex persons with menstrual issues

The findings on clinical encounters show that only a few HCW reported ever attending to an intersex patient with menstrual-related issues, while the vast majority of HCW had never encountered such a case. This is because either intersex individuals rarely seek care for menstrual health needs due to stigma, lack of trust, or invisibility within the health system, or cases may go unrecognised because healthcare workers are not adequately trained to identify intersex variations.

In terms of workplace readiness, when asked about workplace guidelines, only 37 respondents (57%) reported having clear protocols, while 17 (26%) said no and 11 (17%) were unsure.

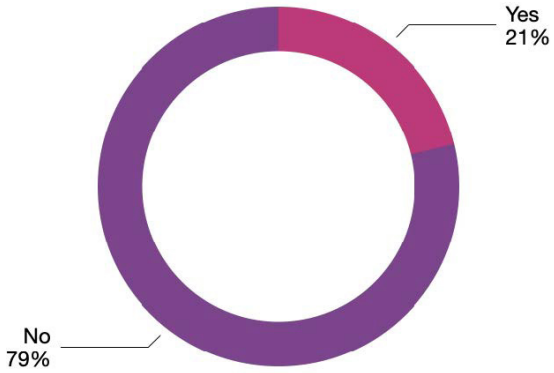


Figure 8: Healthcare workers responses to the availability of workplace guidelines on intersex menstrual health

Confidence in Providing Menstrual Care to Intersex Patients

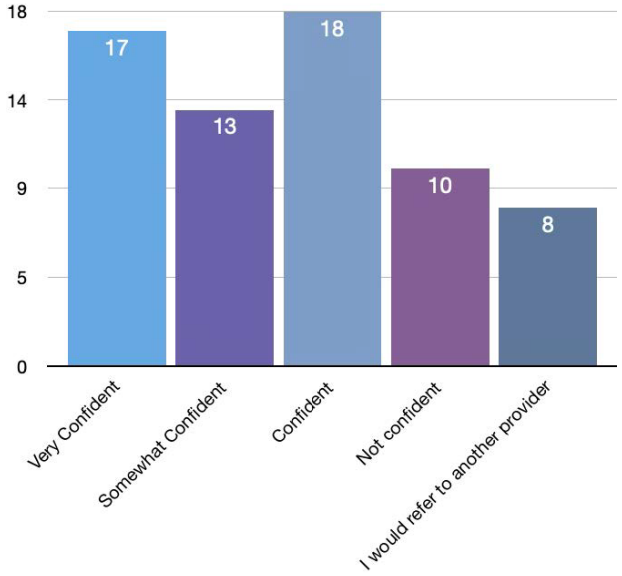


Figure 7: Shows Healthcare workers confidence in providing menstrual care to intersex patients

Even when health care workers have or have not encountered intersex patients, the confidence in providing menstrual care to intersex patients reveals that while a good number of HCW are confident, a few HCWs are not confident, and some would prefer to refer such cases to another provider. Although over half of the respondents expressed confidence in managing intersex menstrual health, a significant portion remain uncertain or unwilling to take on such cases.

Similarly, only 4 HCW (6%) had ever received a clinical protocol or resource on intersex menstrual health. This shows that there is a near-total absence of structured, practical tools within the health system to guide providers in delivering intersex-inclusive care.

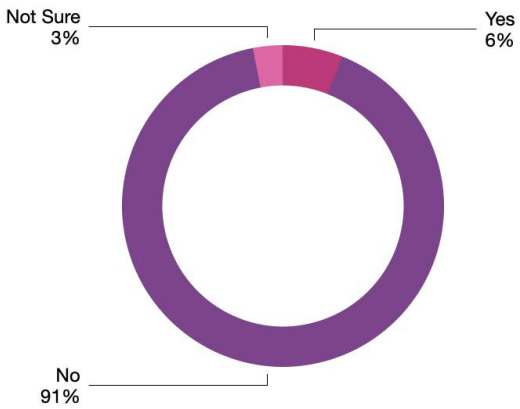


Figure 9: Responses on whether Healthcare workers have received any clinical protocol or resources concerning intersex menstrual health

Barriers to Inclusive Care

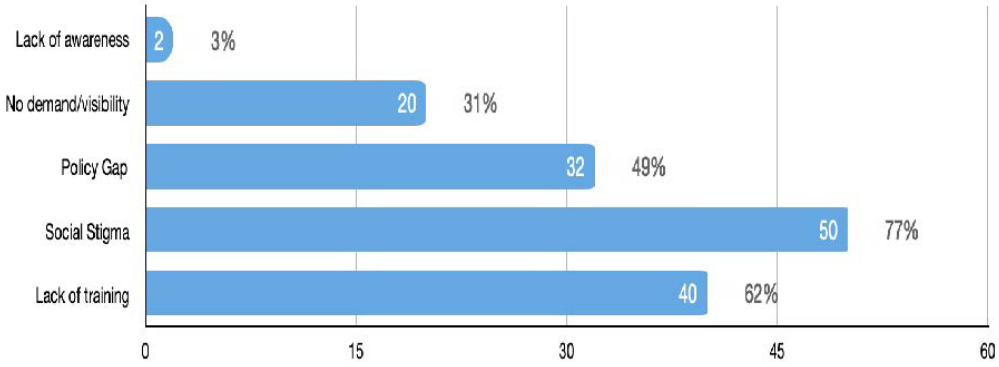


Figure 10: Responses to barriers to intersex inclusion in menstrual health

Social stigma and lack of training of HCW is identified as the most significant challenge to inclusive menstrual health care for intersex persons. Other barriers include low visibility or demand for intersex-specific services and, to a much lesser extent, lack of awareness. The relatively lower emphasis on lack of awareness suggests that while intersex issues are somewhat recognised, the absence of supportive policies and professional capacity remains the primary obstacle to inclusive menstrual health care.

To further understand the barriers to inclusive care for intersex individuals in depth, we explore the interviews where strong emphasis was made by healthcare workers concerning stigma and judgment from providers as a key obstacle, with HCW acknowledging that such attitudes discourage intersex people from seeking care. Additionally, several respondents pointed out that the lack of training and explicit clinical guidelines further contributes to exclusion, as most rely on general SRH protocols that make no mention of intersex-specific needs. This absence of structured direction leaves providers uncertain and hesitant in their practice. One healthcare worker explained that

> ***"If there were clear policies or manuals, it would help us know what to do."***

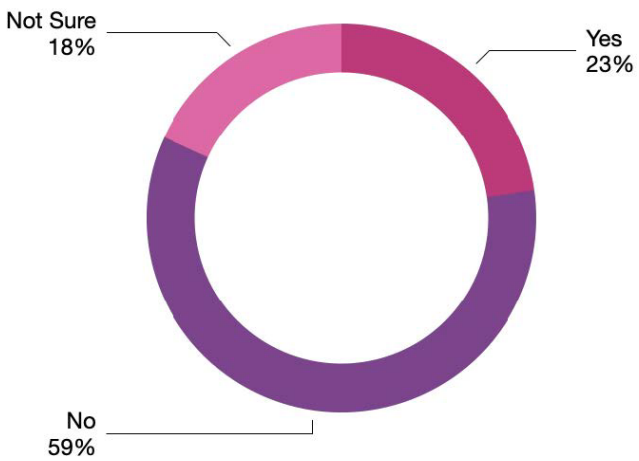


Figure 11: Shows Healthcare workers opinions on whether current SRH services include intersex needs

To explain issues regarding whether current sexual and reproductive health (SRH) services in Nigeria adequately include the needs of intersex individuals, the responses reveal a divided perspective among healthcare workers. While a good number of respondents believed that SRH services do adequately include intersex needs, a considerable proportion of respondents felt that they do not and some other respondents were not sure.

Inclusion and Policy Gaps

When reflecting on menstrual health inclusion, intersex persons consistently emphasised their invisibility and exclusion within existing education, health, and policy frameworks. Schools and healthcare systems are seen as reinforcing binary understandings of menstruation, leaving intersex people without recognition or support. Intersex individuals stressed the importance of reforming education:

> ***"Schools should teach that not only girls menstruate. That would have made me feel normal."***

While there is a need to reform the education sector, there is also a need for capacity-building in the health sector:

> ***"We need health workers to be trained to understand our bodies, not judge us."***

The types of support healthcare workers believe would enable them to provide better care to intersex individuals who menstruate vary, with a strong demand for capacity-building and systemic guidance. The survey shows that the majority of respondents emphasised the need for 28
continuing medical education, mentorship from specialists and the development of national clinical guidelines.

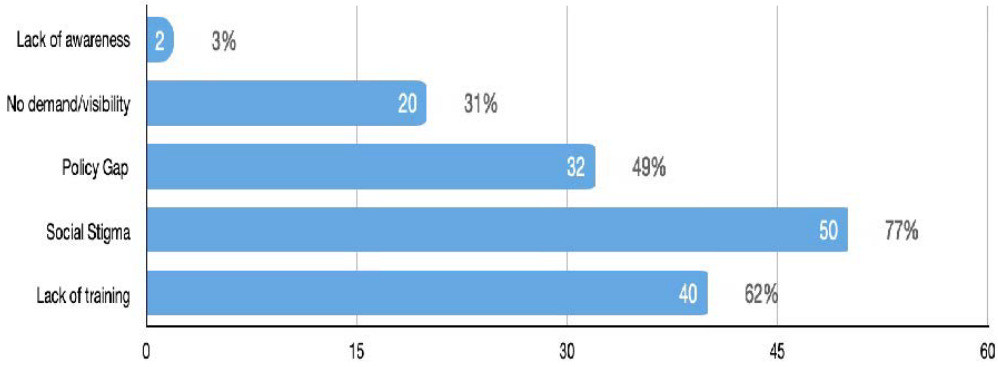


Figure 12: Responses to support needed for better care to intersex individuals who menstruate

The finding shows that there is an urgent need for policy and programmatic reforms that move beyond binary models of menstrual health. Inclusion requires comprehensive strategies that integrate intersex realities into school curricula, healthcare provider training, and national SRH policies, ensuring that intersex individuals are acknowledged, supported, and empowered.

05

Discussion, Conclusions and Recommendations

Discussion of Findings

This study explored the menstrual health needs and experiences of intersex individuals in Nigeria, alongside the knowledge, attitudes, and practices of healthcare workers who provide sexual and reproductive health (SRH) services. The study reveals significant gaps in awareness, training, service delivery, and inclusion, with important psychosocial consequences for intersex people. The findings show that intersex individuals in Nigeria experience menstruation in diverse and often irregular ways. Their first encounters were marked by fear and confusion due to the exclusive framing of menstruation as a girls-only experience. Limited access to accurate information, exclusion from school curricula, and lack of tailored menstrual products further reinforced stigma and secrecy. Psychosocial challenges, including shame, isolation, and secrecy, highlight the absence of community or institutional support.

The study also revealed that healthcare providers are aware of intersex as a concept but lack practical knowledge or tools to manage intersex menstrual health. Only a small fraction had received training or clinical protocols, and many expressed discomfort or preferred to refer cases. Stigma and judgment were frequently acknowledged as barriers, while policy neglect perpetuated invisibility. However, providers expressed willingness to improve, calling for continuing medical education, mentorship, and national guidelines.

Conclusions

This study concludes that intersex individuals remain largely invisible in Nigeria's menstrual health landscape, facing exclusion from education, healthcare, and policy. Interviews with healthcare workers show that while awareness of intersex exists among healthcare providers, their knowledge remains shallow, with most lacking a detailed understanding of the menstrual health needs of intersex individuals. This limited knowledge is compounded by deeply rooted stigma and exclusion, as both social norms and provider-level biases, reinforced by binary sexual and reproductive health (SRH) frameworks, continue to marginalise intersex persons. Healthcare workers, though willing, are underprepared due to systemic neglect, stigma, and lack of resources.

Addressing these challenges requires a comprehensive response that combines policy reform, provider training, stigma reduction, and psychosocial support mechanisms. Furthermore, policy neglect perpetuates invisibility, as intersex-specific issues remain absent from SRH policies and national programming, resulting in systemic exclusion. On a personal and community level, intersex individuals experience profound psychosocial challenges, often marked by secrecy, shame, and isolation, with limited access to peer, family, or institutional support systems. With these gaps identified, there is an urgent need for comprehensive reforms that address training, stigma, policy, and psychosocial support to ensure intersex inclusion in menstrual health.



Recommendations

Policy and Institutional Reform

- The Government should consider integrating intersex needs into national SRH policies explicitly including intersex individuals in sexual and reproductive health (SRH), Water, Sanitation and Hygiene (WASH), and menstrual health frameworks.
- National authorities should develop clinical guidelines and clear protocols on menstrual health management for intersex persons.
- Healthcare facilities adopt inclusive workplace guidelines addressing stigma, patient dignity, and confidentiality.

Health System Strengthening

- Establish regular in-service continuing medical education on intersex health, with a focus on menstrual health.
- Provide user-friendly manuals and decision-support tools to assist providers in delivering inclusive care.

Psychosocial and Community Support

- Support community-based organizations and networks that provide psychosocial support, peer mentorship, and referrals.
- Incorporate mental health services into SRH care, addressing stigma, secrecy, and shame reported by intersex individuals.

Menstrual Health Access and Product Support

- Expand menstrual product programs to include intersex persons, ensuring access to comfortable and safe products.
- Provide tailored counseling on product choices, considering the anatomical and comfort differences among intersex individuals.

Public Awareness and Stigma Reduction

- Launch stigma-reduction campaigns emphasizing that menstruation is not limited to “girls and women” but can also be experienced by intersex persons.
- Develop mass media and community outreach initiatives that normalize bodily diversity and promote acceptance.
- Engage religious, traditional, and community leaders in promoting inclusion and respect for intersex people.

Education and Training

- Revise menstrual health curricula in schools to reflect body diversity and reduce alienation for intersex students.
- Ensure ongoing professional development through interdisciplinary workshops on intersex-inclusive care.

Strategic Partnerships and Advocacy

- Strengthen partnerships between intersex-led groups, feminist movements, and SRHR organizations to amplify visibility.
- Use research findings to engage policymakers, donors, and professional associations in inclusive menstrual health planning.

Call to Action: From Evidence to Equity

Bridging the Gap for Intersex Menstrual Health in Nigeria

The findings of this report present undeniable evidence: intersex individuals in Nigeria are navigating a healthcare system that renders them invisible, education systems that erase their reality, and a society that stigmatizes their bodies.

This research must not remain static on a shelf. It is a tool for transformation. We call upon the following stakeholders and all other interested parties to engage with these findings and take specific, measurable actions to bridge the gap between evidence and advocacy.

1. To the Federal Ministry of Health and Policymakers

- **Acknowledge and Institutionalize:** Officially recognize intersex individuals within the National Sexual and Reproductive Health (SRH) and Menstrual Health Management (MHM) policy frameworks.

  Action: Revise the definition of "menstruators" in national documents to be inclusive of all people who bleed, not just "women and girls".

- **Standardize Clinical Protocols:** The research indicates that 91% of healthcare workers have never received clinical protocols on intersex menstrual health.

  Action: Convene a technical working group, including intersex activists and medical specialists, to draft and disseminate a National Standard Operating Procedure (SOP) for the care of intersex persons, specifically addressing menstrual pain, amenorrhea, and hormonal variations amog other intersex medical requitements and needs.

- **Mandate Data Inclusion:** Address the data gap by including intersex indicators in national health demographic surveys. You cannot plan for a population you do not count.

2. To Healthcare Professional Bodies (NMA, NANNM, SOGON, NMDC)

- **Prioritize Competency-Based Training:** With 83% of surveyed healthcare workers reporting no prior training on intersex health, ignorance is a primary barrier.

  Action: Integrate intersex health modules into the mandatory Continuing Medical Education (CME) and Continuing Professional Development (CPD) curricula for doctors, nurses, and midwives.

- **Create Mentorship Networks:** Healthcare workers identified "mentorship from specialists" (71%) as a top need.

  Action: Establish a referral and mentorship pathway where general practitioners can consult with specialists (endocrinologists, urologists, psychologists) who are trained in rights-based intersex care.

3. To Civil Society Organizations (CSOs) and SRHR Advocates

- **Mainstream, Don't Isolate:** Stop treating intersex issues as a niche topic.

  Action: Review your existing menstrual health programs, WASH campaigns, and period poverty interventions. Ensure your language ("people who menstruate") and your imagery represents gender diversity.

- **Advocate for Product Diversity:** The research highlights that standard pads are often uncomfortable or dysphoria-inducing for intersex persons.

  Action: When conducting product distribution drives, include alternative menstrual products (such as tampons, menstrual cups, or period underwear) and provide education on how to use them.

- **Amplify Intersex Voices:** Use the narratives in this report to challenge the single story of menstruation.

  Action: Partner with Intersex Nigeria to co-host sensitization workshops, ensuring that intersex persons are the ones telling their own stories.

4. To Educational Institutions and Curriculum Developers

- **Break the Binary in Biology:** Intersex participants reported feeling "betrayed" and "sick" because they were taught that menstruation only happens to girls.

  Action: Revise Basic Science and Health Education curricula to accurately teach human biological diversity. Teach students that bodies come in a spectrum, reducing the shock and shame for intersex children reaching puberty.

- **Safe School Environments:**

  Action: Ensure school WASH facilities offer privacy and safety for all students, regardless of their gender presentation, to reduce absenteeism among intersex youth.

5. To Donors and Development Partners

- **Fund the Unseen:** The lack of large-scale data is a hurdle to policy change.

  Action: Provide dedicated funding for large-scale, quantitative prevalence studies and longitudinal research on the health outcomes of intersex Nigerians.

- **Support Psychosocial Interventions:** The study highlights deep isolation and mental health challenges.

  Action: Fund community-led psychosocial support systems, peer support groups, and mental health services tailored specifically for intersex persons.

6. To the Media and Community Gatekeepers

- **Change the Narrative:**

  Action: Move away from sensationalist reporting. Use this report to write evidence-based stories that humanize intersex experiences, debunking myths that intersex traits are "curses" or "bad omens".

- **Normalize Diversity:**

  Action: Use correct terminology. Avoid derogatory labels and help the public understand that intersex variations are a natural part of human biology.

How to Use This Report for Immediate Impact

- **For Advocacy:** Cite the statistics on provider unwillingness and lack of training in grant proposals and policy briefs to justify the need for urgent funding and programming.
- **For Training:** Use the "Lived Experiences" section as case studies in medical training to build empathy and understanding among students.
- **For Community Building:** Share the findings with intersex networks to validate their experiences, proving to them that they are not alone and that their health matters.

The time for silence is over. The time for inclusive action is now.

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website: intersexnigeria.org
email: info@intersexnigeria.org